Gemeinsamer Bundesausschuss

Resolution

of the Federal Joint Committee on an Amendment of the Pharmaceuticals Directive:
Annex XII – Benefit Assessment of Medicinal Products with New Active Ingredients according to Section 35a SGB V
Asciminib (reassessment of an orphan drug > EUR 30 million turnover limit: chronic myeloid leukaemia, Ph+, after ≥ 2 prior therapies)

of 20 November 2025

At their session on 20 November 2025, the Federal Joint Committee (G-BA) resolved to amend the Pharmaceuticals Directive (AM-RL) in the version dated 18 December 2008 / 22 January 2009 (Federal Gazette, BAnz. No. 49a of 31 March 2009), as last amended by the publication of the resolution of D Month YYYY (Federal Gazette, BAnz AT DD.MM.YYYY BX), as follows:

I. **In Annex XII, the information on the active ingredient Asciminib in the version of the resolution of 16 March 2023 shall be replaced by the following information:**

Courtesy translation – only the German version is legally binding.

Asciminib

Resolution of: 20 November 2025
Entry into force on: 20 November 2025
Federal Gazette, BAnz AT DD. MM YYYY Bx

Therapeutic indication (according to the marketing authorisation of 25 August 2022):

Scemblix is indicated for the treatment of adult patients with Philadelphia chromosome-positive chronic myeloid leukaemia in chronic phase (Ph+ CML-CP) previously treated with two or more tyrosine kinase inhibitors.

Therapeutic indication of the resolution (resolution of 20 November 2025):

See therapeutic indication according to marketing authorisation.

1. Additional benefit of the medicinal product in relation to the appropriate comparator therapy

a) Adults with Philadelphia chromosome-positive chronic myeloid leukaemia in chronic phase (Ph+ CML-CP) previously treated with two or more tyrosine kinase inhibitors

Appropriate comparator therapy:

- Individualised therapy with selection of
  - nilotinib,
  - dasatinib,
  - bosutinib and
  - ponatinib

Extent and probability of the additional benefit of asciminib compared to the appropriate comparator therapy:

a1) Adults with Philadelphia chromosome-positive chronic myeloid leukaemia in chronic phase (Ph+ CML-CP) previously treated with two or more tyrosine kinase inhibitors, for whom bosutinib is the appropriate individualised therapy

Indication of a minor additional benefit.

a2) Adults with Philadelphia chromosome-positive chronic myeloid leukaemia in chronic phase (Ph+ CML-CP) previously treated with two or more tyrosine kinase inhibitors, for whom nilotinib, dasatinib or ponatinib is the appropriate individualised therapy

An additional benefit is not proven.

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Study results according to endpoints:[1]

a1) Adults with Philadelphia chromosome-positive chronic myeloid leukaemia in chronic phase (Ph+ CML-CP) previously treated with two or more tyrosine kinase inhibitors, for whom bosutinib is the appropriate individualised therapy

Summary of results for relevant clinical endpoints

Endpoint category	Direction of effect/ risk of bias	Summary
Mortality	↔	No relevant difference for the benefit assessment
Morbidity	↑	Advantages for the endpoints of symptom severity and impairment of daily life due to symptoms (assessed using MDASI-CML)
Health-related quality of life	∅	No data available.
Side effects	↑↑	Advantages in the endpoints of SAEs, severe AEs (CTCAE grade ≥ 3) and discontinuation due to AEs. In detail, mainly advantages in specific AEs.
Explanations: ↑: statistically significant and relevant positive effect with low/unclear reliability of data ↓: statistically significant and relevant negative effect with low/unclear reliability of data ↑↑: statistically significant and relevant positive effect with high reliability of data ↓↓: statistically significant and relevant negative effect with high reliability of data ↔: no statistically significant or relevant difference ∅: No data available. n.a.: not assessable		

ASCEMBL study:

- Completed, multicentre, open-label RCT
- Asciminib versus bosutinib

Data cut-offs:

- 22 March 2023: 30 days after the end of the study treatment
- 4 December 2024: final analysis of overall survival and progression-free survival

[1] Data from the dossier assessment of the IQWiG (A25-70) and from the addendum (A25-129), unless otherwise indicated.

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Mortality

Endpoint	Asciminib		Bosutinib		Asciminib vs bosutinib
	N	Median time to event in months [95% CI] *Patients with event n (%)*	N	Median time to event in months [95% CI] *Patients with event n (%)*	HR [95% CI]; p value
Overall survival[a]	157	n.r.[b] *16 (10.2)*	76	n.r. [75.72; n.c.][b] *9 (11.8)*	0.79 [0.35; 1.78]; 0.564

Morbidity

Endpoint	Asciminib		Bosutinib		Asciminib vs bosutinib
	N	Median time to event in months [95% CI] *Patients with event n (%)*	N	Median time to event in months [95% CI] *Patients with event n (%)*	HR [95% CI]; p value
Progression to the blast phase[a]					
	157	–[c] *5 (3.2)*	76	–[c] *5 (6.6)*	0.37 [0.09; 1.50]; 0.146
Symptomatology[d]					
PGIC – time to 1st deterioration[e]	157	n.r. *19 (12.1)*	76	n.r. *13 (17.1)*	0.58 [0.29; 1.19]; 0.138
MDASI-CML[d]					
Symptom severity (total symptom severity – time to 1st deterioration[f])	152	n.r. *19 (12.5)*	70	n.r. [22.11; n.c.] *21 (30.0)*	0.31 [0.17; 0.59]; < 0.001
Impairment of everyday life due to symptoms (symptom interference – time to 1st deterioration[f])	152	n.r. *36 (23.7)*	70	n.r. [5.95; n.c.] *24 (34.3)*	0.56 [0.33; 0.94]; 0.029
Health status					
EQ-5D VAS – time to 1st deterioration[g]	150	n.r. *46 (30.7)*	69	n.r. [3.68; n.c.] *24 (34.8)*	0.77 [0.47; 1.27]; 0.300

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Health-related quality of life

No data available.

Side effects[d]

Endpoint	Asciminib		Bosutinib		Asciminib vs bosutinib
	N	Median time to event in months [95% CI] *Patients with event n (%)*	N	Median time to event in months [95% CI] *Patients with event n (%)*	HR [95% CI]; p value; Absolute difference (AD)[h]
Total adverse events (presented additionally)					
	156	0.41 [0.26; 0.95] *142 (91.0)*	76	0.08 [0.03; 0.20] *74 (97.4)*	–
Serious adverse events (SAE)					
	156	n.r. [50.10; n.c.] *34 (21.8)*	76	n.r. [25.10; n.c.] *20 (26.3)*	0.53 [0.30; 0.94]; 0.027
Severe adverse events[i]					
	156	9.26 [3.25; 21.19] *93 (59.6)*	76	3.48 [1.84; 8.31] *52 (68.4)*	0.69 [0.49; 0.98]; 0.033; AD = 5.78
Discontinuation due to adverse events					
	156	n.r. *13 (8.3)*	76	n.r. [25.10; n.c.] *21 (27.6)*	0.20 [0.10; 0.41]; < 0.001
Specific adverse events					
Respiratory, thoracic and mediastinal disorders (SOC, severe AEs[i])	156	n.r. *1 (0.6)*	76	n.r. [32.62; n.c.] *5 (6.6)*	0.04 [0.00; 0.34]; < 0.001
Gastrointestinal disorders (SOC, severe AEs[i])	156	n.r. *8 (5.1)*	76	n.r. *12 (15.8)*	0.22 [0.09; 0.55]; < 0.001
Skin and subcutaneous tissue disorders (SOC, severe AEs[i])	156	n.r. *1 (0.6)*	76	n.r. *8 (10.5)*	0.06 [0.01; 0.45]; < 0.001
Alanine aminotransferase elevated (PT, severe AEs[i])	156	n.r. *1 (0.6)*	76	n.r. *11 (14.5)*	0.04 [0.01; 0.31]; < 0.001
Aspartate aminotransferase	156	n.r. *3 (1.9)*	76	n.r. *5 (6.6)*	0.23 [0.05; 0.95]; 0.027

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elevated (PT, severe AEs[i])					
Thrombocytopenia (PT, severe AEs[i])	156	n.r. *28 (17.9)*	76	n.r. *5 (6.6)*	2.79 [1.08; 7.23]; 0.027

[a] Data cut-off from 4 December 2024
[b] Calculation by IQWiG from data in years (years x 12)
[c] Median time [95% CI] to event not meaningfully interpretable
[d] Data cut-off from 22 March 2023
[e] Reaching a scale value of 5, 6 or 7 is considered as clinically relevant deterioration (scale range: 1 to 7).
[f] An increase in score by ≥ 1.5 points compared to the start of the study is considered as clinically relevant deterioration (scale range: 0 to 10 points).
[g] A decrease by ≥ 15 points compared to the start of the study is considered as clinically relevant deterioration (scale range: 0 to 100).
[h] Indication of absolute difference (AD) only in case of statistically significant difference; own calculation
[i] Operationalised as CTCAE grade ≥ 3

Abbreviations used:
AD = absolute difference; CTCAE = Common Terminology Criteria for Adverse Events; HR = hazard ratio; CI = confidence interval; N = number of patients evaluated; n = number of patients with (at least one) event; n.c. = not calculable; n.r. = not reached; vs = versus

a2) <u>Adults with Philadelphia chromosome-positive chronic myeloid leukaemia in chronic phase (Ph+ CML-CP) previously treated with two or more tyrosine kinase inhibitors, for whom nilotinib, dasatinib or ponatinib is the appropriate individualised therapy</u>

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Summary of results for relevant clinical endpoints

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Health-related quality of life	∅	No data available.
Side effects	∅	No data available.

Explanations:
↑: statistically significant and relevant positive effect with low/unclear reliability of data
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↑↑: statistically significant and relevant positive effect with high reliability of data
↓↓: statistically significant and relevant negative effect with high reliability of data
↔: no statistically significant or relevant difference
∅: No data available.
n.a.: not assessable

2. Number of patients or demarcation of patient groups eligible for treatment

Adults with Philadelphia chromosome-positive chronic myeloid leukaemia in chronic phase (Ph+ CML-CP) previously treated with two or more tyrosine kinase inhibitors

Approx. 1,500 to 1,730 patients

3. Requirements for a quality-assured application

The requirements in the product information are to be taken into account. The European Medicines Agency (EMA) provides the contents of the product information (summary of product characteristics, SmPC) for Scemblix (active ingredient: asciminib) at the following publicly accessible link (last access: 07 August 2025):

https://www.ema.europa.eu/en/documents/product-information/scemblix-epar-product-information_en.pdf

Treatment with asciminib should only be initiated and monitored by specialists in internal medicine, haematology and oncology experienced in the treatment of patients with chronic myeloid leukaemia.

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4. Treatment costs

Annual treatment costs:

The costs for the first year of treatment are shown for the cost representation in the resolution.

Adults with Philadelphia chromosome-positive chronic myeloid leukaemia in chronic phase (Ph+ CML-CP) previously treated with two or more tyrosine kinase inhibitors

Designation of the therapy	Annual treatment costs/ patient
Medicinal product to be assessed:	
Asciminib	€ 60,350.97
Appropriate comparator therapy:	
Bosutinib	€ 24,555.38
Dasatinib	€ 9,517.86
Nilotinib	€ 47,835.34
Ponatinib	€ 76,811.57

Costs after deduction of statutory rebates (LAUER-TAXE® as last revised: 15 September 2025)

Costs for additionally required SHI services: not applicable

5. Designation of medicinal products with new active ingredients according to Section 35a, paragraph 3, sentence 4 SGB V that can be used in a combination therapy with the assessed medicinal product

In the context of the designation of medicinal products with new active ingredients pursuant to Section 35a, paragraph 3, sentence 4 SGB V, the following findings are made:

Adults with Philadelphia chromosome-positive chronic myeloid leukaemia in chronic phase (Ph+ CML-CP) previously treated with two or more tyrosine kinase inhibitors

– No medicinal product with new active ingredients that can be used in a combination therapy that fulfils the requirements of Section 35a, paragraph 3, sentence 4 SGB V.

The designation of combinations exclusively serves the implementation of the combination discount according to Section 130e SGB V between health insurance funds and pharmaceutical companies. The findings made neither restrict the scope of treatment required to fulfil the medical treatment mandate, nor do they make statements about expediency or economic feasibility.

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II. **The resolution will enter into force on the day of its publication on the website of the G-BA on 20 November 2025.**

The justification to this resolution will be published on the website of the G-BA at www.g-ba.de.

Berlin, 20 November 2025

Federal Joint Committee (G-BA)
in accordance with Section 91 SGB V
The Chair

Prof. Hecken

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